

--SUMMARY--

Decision No. 815/17

12-Apr-2017

C.Ramsay - M.Lipton - S.Roth

- Continuing entitlement

No Summary Available

18 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w2417n]

Style of Cause:

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WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 815/17

BEFORE:

C.J. Ramsay : Vice-Chair
M. Lipton : Member Representative of Employers
S. Roth : Member Representative of Workers

HEARING:

March 13, 2017 at London
Oral

DATE OF DECISION:

April 12, 2017

NEUTRAL CITATION:

2017 ONWSIAT 1096

DECISION UNDER APPEAL:

WSIB Appeals Resolution Officer (ARO) February 11, 2014

APPEARANCES:

For the worker:

H. Kamal, Paralegal

For the employer:

Not participating

Interpreter:

G. Alathamana, Arabic

REASONS

(i) Introduction

- [1] The worker appeals a decision of the ARO, which concluded that the worker had fully recovered from injuries to her right shoulder and head that occurred on September 28, 2007. The ARO also concluded that the worker had fully recovered from injuries sustained to her back and right shoulder that occurred on December 2, 2007. The ARO rendered a decision based upon the written record without an oral hearing.

(ii) Issues

- [2] The issues under appeal are as follows:
1. Whether the worker is entitled to ongoing entitlement for injuries sustained on September 28, 2007;
 2. Whether the worker is entitled to ongoing entitlement for injuries sustained on December 2, 2007;
 3. Whether the worker is entitled to further loss of earnings (“LOE”) benefits beyond June 1, 2008;
 4. Whether the worker has a work-related permanent impairment of her head, neck, or lower back for which she is entitled to a non-economic loss (“NEL”) determination.

(iii) Background

- [3] The following are the basic facts.
- [4] The now 48 year old worker started as a customer service representative with the accident employer in 2007. She injured her head and right shoulder on September 28, 2007 when she slipped and fell at work. The Board accepted the worker’s claim and full LOE benefits were paid from October 19, 2007 to November 26, 2007.
- [5] The worker returned to work with the accident employer on a modified basis. On December 2, 2007, the worker slipped and fell on the accident employer’s premises. The Board accepted the worker’s claim and paid LOE benefits to June 1, 2008. The Board determined that the worker had been offered suitable work at no wage loss as of June 1, 2008, so LOE benefits were closed.
- [6] The Board determined that the worker would fully recover as of June 28, 2008 from the injuries sustained on December 2, 2007 in a decision dated May 30, 2008. The Board subsequently determined that the worker fully recovered from both injuries without permanent impairment in a decision dated September 19, 2008.
- [7] The worker objected to the closure of LOE benefits as well as the decisions that the worker fully recovered and that she was not permanently impaired. The ARO denied the worker’s appeals.

(iv) Law and policy

[8] Since the worker sustained injuries in 2007, the *Workplace Safety and Insurance Act, 1997* (the WSIA) is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[9] Specifically, sections 40 and 43 of the WSIA govern the worker's entitlement in this case. Section 40 of the WSIA provides in part:

40(1) The employer of an injured worker shall co-operate in the early and safe return to work of the worker by,

- (a) contacting the worker as soon as possible after the injury occurs and maintaining communication throughout the period of the worker's recovery and impairment;
- (b) attempting to provide suitable employment that is available and consistent with the worker's functional abilities and that, when possible, restores the worker's pre-injury earnings;
- (c) giving the Board such information as the Board may request concerning the worker's return to work; and
- (d) doing such other things as may be prescribed. 1997, c. 16, Sched. A, s. 40 (1).

(2) The worker shall co-operate in his or her early and safe return to work by,

- (a) contacting his or her employer as soon as possible after the injury occurs and maintaining communication throughout the period of the worker's recovery and impairment;
- (b) assisting the employer, as may be required or requested, to identify suitable employment that is available and consistent with the worker's functional abilities and that, when possible, restores his or her pre-injury earnings;
- (c) giving the Board such information as the Board may request concerning the worker's return to work; and
- (d) doing such other things as may be prescribed. 1997, c. 16, Sched. A, s. 40 (2).

...

[10] Section 43 of the WSIA provides in part that:

43(1) A worker who has a loss of earnings as a result of the injury is entitled to payments under this section beginning when the loss of earnings begins. The payments continue until the earliest of,

- (a) the day on which the worker's loss of earnings ceases;
- (b) the day on which the worker reaches 65 years of age, if the worker was less than 63 years of age on the date of the injury;
- (c) two years after the date of the injury, if the worker was 63 years of age or older on the date of the injury;
- (d) the day on which the worker is no longer impaired as a result of the injury. 1997, c. 16, Sched. A, s. 43 (1).

...

(3) The amount of the payment is 85 per cent of the difference between his or her net average earnings before the injury and any net average earnings the worker earns after the injury, if the worker is co-operating in health care measures and,

- (a) his or her early and safe return to work; or
 - (b) all aspects of a labour market re-entry assessment or plan. 1997, c. 16, Sched. A, s. 43 (3); 2000, c. 26, Sched. I, s. 1 (6).
- (4)** The Board shall deem the worker's earnings after the injury to be the earnings that the worker is able to earn from the employment or business that is suitable for the worker under section 42 and,
- (a) if the worker is provided with a labour market re-entry plan, the earnings shall be deemed as of the date the worker completes the plan; or
 - (b) if the Board determines that the worker does not require a labour market re-entry plan, the earnings shall be deemed as of the date this determination is made. 1997, c. 16, Sched. A, s. 43 (4).
- ...
- (7)** The Board may reduce or suspend payments to the worker during any period when the worker is not co-operating,
- (a) in health care measures;
 - (b) in his or her early and safe return to work; or
 - (c) in all aspects of a labour market re-entry assessment or plan provided to the worker. 1997, c. 16, Sched. A, s. 43 (7).

[11] As noted above, one of the issues before the Tribunal is the worker's entitlement to LOE benefits. Under section 43(1) a worker who has a loss of earnings as a result of a compensable injury is entitled to LOE benefits. *Decision No. 2474/00* held that under section 43(1) a causal relationship between the injury and wage loss is a condition precedent to the payment of LOE benefits. A refusal of suitable work is not necessarily an act of non-cooperation, but it may lead to a conclusion that the worker's loss of earnings does not result from the injury. Section 43(2) operates to reduce a worker's benefits where the worker refuses suitable employment. Thus, a worker who refuses suitable employment at no wage loss is not entitled to LOE benefits because the loss of earnings is not caused by the injury, but caused by the refusal of the suitable employment.

[12] Section 46 of the *Workplace Safety and Insurance Act, 1997* (the WSIA) and section 42 of the pre-1997 *Workers' Compensation Act*, as amended, provide that if a worker's injury results in permanent impairment, the worker is entitled to compensation for non-economic loss.

[13] "Impairment" means a physical or functional abnormality or loss (including disfigurement) which results from an injury and any psychological damage arising from the abnormality or loss.

[14] "Permanent impairment" means impairment that continues to exist after the worker reaches maximum medical recovery ("MMR").

[15] Legislation and Board policy provide that the degree of a worker's permanent impairment is determined in accordance with the prescribed rating schedule or criteria, any medical assessments, and having regard to the health information on file. The prescribed rating schedule for most impairments is the American Medical Association's *Guides to the Evaluation of Permanent Impairment*, 3rd edition (revised) (the AMA Guides). The Board has adopted specific rating schedules for impairment due to psychological disability, fibromyalgia, chronic pain and other conditions.

- [16] Tribunal jurisprudence applies the test of significant contribution to questions of causation. A significant contributing factor is one of considerable effect or importance. It need not be the sole contributing factor. See, for example, *Decision No. 280*.
- [17] The standard of proof in workers' compensation proceedings is the balance of probabilities.
- [18] Pursuant to section 126 of the WSIA, the Board stated that the following policy packages, Revision #9, would apply to the subject matter of this appeal:
- Policy Package(s): 33; 61; 300**
- Package # 33 - LOE Benefits - benefits prior to December 1, 2010**
- 18-03-02 - Payment of LOE Benefits, dated August 1, 2007
- Package # 61 - NEL Entitlement - Decisions prior to March 30, 2011**
- 11-01-05 - Determining Maximum Medical Recovery (MMR)
- 18-05-03 - Determining the Degree of Permanent impairment, dated July 18, 2008
- Package # 300 - Decision Making/Benefit of Doubt/Merits and Justice**
- 11-01-02 - Decision-Making
- 11-01-03 - Merits and Justice
- 11-01-13 - Benefit of Doubt
- [19] We have considered these policies as necessary in deciding the issues in this appeal.

(v) Testimony

- [20] The worker testified that she came to Canada in 1997. She testified that she received a law degree in her home country. She testified that she worked as a lawyer for 2 to 3 years in her home country before coming to Canada.
- [21] The worker testified that she took English as a second language courses when she came to Canada. She testified that she first started working in 2007. She testified that she started working with the accident employer in September 2007 as a cashier working at the front desk.
- [22] The worker testified that her regular duties consisted of two different tasks: taking orders or making orders. She testified that people that take orders still were required to make coffee. She testified that people that made orders would make other things such as sandwiches, bagels and other drinks.
- [23] The worker testified that on September 28, 2007 she was working at the cash taking orders. She testified that somebody was mopping behind her. She testified that she did not see that the floor was wet and she slipped and fell and landed on her right side. She testified that she lost consciousness for about a minute. She testified that her husband picked her up and took her to the hospital. She testified that the next day her face was swollen and bruised. She testified that the bruise lasted for a month.
- [24] The worker testified that she started having headaches right after the fall. She testified that her head felt as if it was going to explode.

- [25] The worker testified that she did not recall how long she was off work. She testified that she returned to work with the accident employer. She testified that the duties were not really modified other than she was no longer making orders. She testified that she had difficulty with the modified duties because the cash register was radiating and making her head hurt and her eyes blurry.
- [26] The worker testified that she suffered a second injury at work on December 2, 2007. She testified that she went into work and was not feeling well. She testified that the cash register affected her headaches. She testified that there were enough people at work so she was sent home early. She testified that when she was walking to her car in the parking lot she slipped and fell. She testified that she immediately had pain in her right hand and right shoulder due to the fall. She testified that her husband took her to the hospital.
- [27] The worker testified that she has undergone physiotherapy since the two accidents. She testified that physiotherapy gave her temporary relief.
- [28] The worker testified that since the accident she has had constant headaches, pain in her back, neck, shoulder and is difficult for her to concentrate. She testified that her shoulder hurts if she sits for longer than 15 minutes and the pain shoots down her shoulder into her fingers. She testified that she has lots of pain in her back and cannot sit or stand for prolonged periods of time and has to lie down. She testified that since the accident she has been taking Advil, Tylenol 3 and regular Tylenol.
- [29] The worker testified that she did not look for work between 2007 and 2012. The worker testified that she returned to working as a lunchtime supervisor in 2012. She testified that she worked for one hour per day for one year.
- [30] The worker testified that she discontinued working as a lunchtime supervisor because she found it difficult to deal with the noise which exacerbated her headaches. She testified that she returned to school in 2013 to become an early childhood educator. She testified that she went to school for three months on a full-time basis. She testified that she did not complete the program because she found it too difficult to study.
- [31] The worker testified that she was in a motor vehicle accident in 2016 which exacerbated her head, neck, shoulder and back problems.
- [32] The worker testified that she does some shopping with her daughters and some light cleaning. She testified that her husband does the balance of the housekeeping.
- [33] The worker testified that she began seeing a psychologist and psychiatrist in 2013 due to symptoms of depression.
- [34] The worker testified that she had back pain from 2001 until 2003 due to pregnancy. She testified that the back pain resolved prior to her first accident in 2007.

(vi) Submissions

- [35] The worker's representative submitted that the worker should be entitled to a NEL determination for her head because the medical documentation and the worker's testimony substantiate that the worker suffered a head injury after her slip and fall in 2007 which did not resolve. She submitted that the worker has had constant headaches since 2007 and there is no medical evidence which demonstrates that the worker's headaches resolved.

[36] The worker's representative submitted that the worker suffered a strain on her back, neck and rotator cuff, as a result of her two accidents at work. She submitted that the medical documentation shows that the worker's conditions never got better and that the worker continued to have limitations for over a year after the accidents. She submitted therefore that the worker is entitled to NEL determinations for her back, neck, and right shoulder.

[37] The worker's representative submitted that the worker is entitled to loss of earnings benefits up until at least 2016 because the worker was not capable of returning to work with the accident employer in either a modified capacity or to her regular duties.

[38] The worker's representative submitted that the worker had ongoing limitations after 2007 which would have prevented her from returning to work. She submitted that the worker had a poor work history and is not capable of being employed in any other capacity due to her compensable injuries. She submitted that the worker is entitled to full loss of earnings benefits based on her wage with the accident employer.

(vii) Documentary and medical evidence

[39] The Panel has reviewed all of the medical evidence before it, but notes the following relevant medical evidence, in particular, below.

[40] According to a physiotherapy report dated March 12, 2003, the worker advised of a two year history of lower back pain radiating to the iliac spine bilaterally.

[41] According to a Form 8 completed for the Board by Dr. Dukelow, emergency room physician, dated September 28, 2007, the accident history was noted as follows:

Fell from own feet to floor – slipped; hit right brow.

[42] The Form 8 indicated that worker was diagnosed with a minor head injury. The Form noted that the worker was to be off for 24 hours, and then could return to work.

[43] According to a Form 7 completed by the accident employer on October 2, 2007, the worker's regular working hours were 6 a.m. to 11 a.m.

[44] Dr. Wang, family physician, completed a functional abilities form ("FAF") on October 5, 2007. The FAF indicated that the worker had sustained injuries to her right facial bones, and her right shoulder and upper arm. The FAF also indicated that the worker was restricted from lifting more than 5 lbs. for the next five weeks.

[45] Dr. Wang completed another FAF on October 14, 2007 which indicated that the worker was completely restricted from working for the next six weeks.

[46] The accident employer provided the following offer of modified duties by letter dated October 15, 2007:

In cooperation with the WSIB, [the employer] is able to offer you modified work.

This offer is intended to allow you to return to work safely.

The modified work described below is designed to suit your current physical capabilities, so as not to aggravate your injury.

In accordance with the right shoulder and right side of neck soft tissue injury we would like to offer you the following modified work. We expect [the worker] to use her left arm instead of her right for all the tasks described.

JOB DUTIES:

- Completing her CD Rom training
- separating coffee filters (standing or sitting)
- Preparing yogurts and restocking dessert showcase
- Preparing chicken, egg salad
- Portioning Swiss, cheddar and processed cheeses
- Cleaning tables and putting dishes through dishwasher
- Sweeping kitchen
- Sweeping store front
- Steam cleaning in the kitchen
- Cleaning bathrooms
- General cleaning duties wiping chairs, cleaning glass doors, wiping ledges....
- Training for baking (mostly watching)

[47] The worker was referred for physiotherapy treatment. According to a FAF from the worker's physiotherapist dated November 6, 2007, the worker was able to stand for 15-30 minutes, sit for 10-15 minutes, lift up to 5 kgs on an occasional basis and push/pull 10 lbs. on an occasional basis. The worker was not permitted to use a ladder, drive, work above shoulder level, or bend/twist her neck on a sustained basis. It was recommended that the worker could return to work starting at five hours per day.

[48] Dr. Wang completed a report dated November 22, 2007 which indicated that the worker may resume working on November 26, 2007 on a graduated basis starting with two hours of work per day for two weeks. He indicated that the worker was to be reassessed once she had reached working four hours per day for two weeks.

[49] The worker's physiotherapist completed a progress note dated November 29, 2007 which indicated that the worker had returned to work on modified duties and hours. The report further indicated that the worker was still very limited in neck extension and flexion; wrist movement was pain free; still some pain with her elbow, and shoulder motion was limited by pain. The worker's physiotherapist requested additional treatment as of November 30, 2006 to alleviate neck pain, increase strength in the worker's shoulder, and help the worker return to full duties.

[50] Dr. Wang completed a Form 8 for the Board on December 11, 2007. According to the Form, the worker reported that she fell on her back and right hand. The diagnosis was noted to be "sprain/strain, low back & upper right extremity." The worker was referred for physiotherapy.

[51] The worker was referred to Dr. Kay, neurologist. His report dated January 22, 2008 indicated that the worker was referred for a headache due to the September 2007 fall. The report went on to note:

This 39-year-old woman slipped and fell in September 2007, striking her head on her right anterior temple. She may have had a momentary loss of consciousness. She had swelling and bruising around her right eye and the right side of her face. She was seen in the ER on more than one occasion, and had a CT scan.

She has had headaches ever since. The headaches are located above the right eye. The pain consists of a steady squeezing pain with intermittent sharp pain. The pain radiates to the right cheek and from the right fronto-temporal region to the top of her head on the right. She cannot lie comfortably on her right side without developing a throbbing sensation on the right side of her head. The pain has occurred on a daily basis, and is present from the moment she awakens in the morning. The pain was worse after she fell again in December and landed on her low back.

The pain worsened during a respiratory tract infection. The pain is associated with phonophobia but not photophobia, and she has not had nausea. The headache drains her energy level, and she has difficulty completing her chores at home. She has not worked except for 1 week since September. The headache is rated at 7-8 out of 10 in severity each morning, and decreases with Advil or Tylenol to 3/10 in severity.

She has not had any numbness or weakness in her face or limbs. There has been no change in her senses of taste, smell or hearing. She had some diplopia when looking down to read, but this has improved. She has not had any cognitive changes.

[52] The report went on to note:

Assessment: She presents with chronic daily headaches for 4 months following a fall in which she bumped her head. The pain is localized to the area in which she struck her head. She still has some supra-orbital tenderness, and her examination suggests some mild sensory change in the supra-orbital distribution. The numbness may simply be a part of her experience of pain, but it might also indicate that there could have been some trauma to the right supra-orbital or right zygomaticotemporal superficial cutaneous nerves. Injury to one of those nerves could be the reason for her persistent pain.

I did not see any signs to suggest intra-cranial trauma. The normal CT scan is reassuring.

I recommended that she take a headache-preventative medication that has activity against neuropathic pain. I prescribed Nortriptyline starting at 10 mg/d with instructions to increase the dose to 30 mg/d. I discussed the possible side effects of the medication, and indicated that it could take several weeks for benefit to be seen.

[53] The worker's physiotherapist provided an assessment report dated February 16, 2008 which noted that the worker had restrictions with bending, lifting, prolonged standing, and prolonged sitting.

[54] A FAF dated March 7, 2008 completed by the worker's physiotherapist indicated that the worker was restricted to standing for less than 30 minutes, sitting for 30 minutes, and using her hands at her own pace. The worker was completely restricted from lifting, and working above or below her shoulder level, and from performing repetitive duties with her right arm.

[55] Dr. Kay saw the worker again in March 2008. He indicated in his report dated March 18, 2008 that the worker continued to have headaches that were located on the right side, above and around the right eye. He indicated that the headaches were associated with facial numbness. He went on to note:

Assessment:

1. She presents with chronic daily headaches for 6 months following a fall in which she bumped her head.
2. She was intolerant of Nortriptyline.
3. I recommended that she try another headache-preventative medication that has activity against neuropathic pain. I prescribed Gabapentin starting at 300 mg/d with instructions to increase the dose to 300 mg QID. I discussed the off label status of

Gabapentin in its use for pain, the possible side effects of the medication, and indicated that it could take several weeks for benefit to be seen.

[56] The worker was referred for a multi-specialty health care assessment. The assessment was completed by Dr. Bhardwaj and a physiotherapist in April 2008. The report dated April 11, 2008 indicated:

DIAGNOSES:

History of left-sided shoulder, lumbar, and right leg pain and headaches.

History of pre-existing back pain requiring CT scan of the lumbar spine in April of 2007, and x-rays of the SI joints in 2003.

RECOMMENDATIONS:

To date she has had appropriate investigations.

She has had physiotherapy treatments at different physiotherapy facilities. Further hands-on physiotherapy is not necessary; however she can continue with a home exercise program.

Current limitations are to alternate sit, stand, and walk, limit work below the shoulder level, limit lifting to 5 to 10 pounds, limit repetitive bending. She should start work for two hours a day and increase by one hour each week until working full time, followed by a gradual return to regular duties.

[57] The employer made the following offer of modified duties to the worker by letter dated May 1, 2008:

In cooperation with the WSIB, [the accident employer] is able to offer you modified work.

This offer is intended to allow you to return to work safely. The modified work described below is designed to suit your current physical capabilities, so as not to aggravate your injury, as per the precautions and recommendations from the assessment done on April 10th 2008 by the Regional Evaluation Centre.

The following tasks are broken down to standing and sitting duties, these can be alternated as needed, you're not to do any work above shoulder level, lifting is limited to 10lbs and bending stooping at waist and limited twisting.

Standing Job Duties

- Entering orders into a cash register terminal (counter or OT)
- Taking payment from customer
- Giving change back to customer
- General cleaning duties (sweeping inside and outside, cleaning and dusting below shoulder level and above bending past waist.
- Cleaning tables and putting dishes through dishwasher
- Separating coffee and tea filters
- Portioning Cheeses and cold meats
- Scooping cream cheeses

Sitting Job Duties

- Separating coffee and tea filters
- Portioning Cheeses and cold meats

- Scooping cream cheeses

[58] The letter went on to note that the duties were available as of May 5, 2008 and would start at two hours per day and an hour would be added each week until the worker had returned to her pre-accident hours.

[59] Dr. Kay saw the worker again in May 2008. He noted the same observations in his assessment as was noted in his March 2008 report, except he noted that the worker had some improvement in her symptoms with the use of Gabapentin.

[60] Dr. Wang provided a report dated June 18, 2008 which indicated that the worker attended physiotherapy at two different locations. The report went on to note:

She had been off work from September 28, 2007-November 26, 2007 when she returned to work. She was requested to work for only two hours a day, but she was asked by her employer to work five hours a day.

On December 2, 2007 as she was leaving work, she slipped on ice in the parking lot, and this aggravated her already existing pain on her neck, shoulder, arm and leg. This time she had injury to her low back. Incidentally she does have a preexisting low back injury in year 2003. This second fall on December 2nd brought her low back problem to the surface. She has been having back problems ever since. She had altogether two falls at work. One on September 11, 2007, and the other one December 2, 2007.

These two falls give rise to job related injuries to the right side of the face, right head, right shoulder, right arm, right leg, and low back.

The disability has persisted right until the present time; she has not been making any good progress, in fact in some areas the disability is getting somewhat worse.

[61] The worker underwent an MRI of her head on July 17, 2008. The MRI report indicated:

IMPRESSION: Slit-like structure in the left occipital lobe, that is prominent, likely represents asymmetric sulcus which is favoured, versus previous trauma.

Focal enlargement of the A-comm. A tiny aneurysm cannot be excluded at this region.

[62] Dr. Wang completed a Form 26 Progress Report on August 5, 2008. He indicated that the worker had low back pain, right sciatica, muscle spasm, decreased range of motion, pain and tenderness in the back of her neck, and right upper extremity and headaches. The Form 26 indicated that the worker was unable to do household chores.

[63] Dr. Kay saw the worker again in September 2008. His report dated September 11, 2008 indicated:

Assessment:

1. She has ongoing chronic daily headaches.
2. I suggested a trial of Lyrica starting at 75 mg once daily and increasing to 150 mg BID.
3. She had an MRI at [the hospital]. I would like to review the report.

[64] Dr. Kay saw the worker again in December 2008. His report dated December 3, 2008 indicated:

Assessment:

1. She presents with chronic daily headaches following a September 2007 fall in which she bumped her head.

2. She was intolerant of Nortriptyline. She had modest benefit on Gabapentin. She did not fill a prescription for Lyrica.
3. MRI did not show any structural intracranial cause for headache.
4. The focal enlargement of the A-comm should have follow-up. She indicated today that no other tests have been requested yet. I will request a CT-angiogram for May 2009 and I will see her in follow-up after that test.

[65] The worker was involved in a non-work related motor vehicle accident in March 2016. According to the worker's disability certificate dated March 19, 2016 the worker reported suffering injuries to her legs, knees, neck and head.

(viii) Tribunal Discussion Papers

[66] The case materials contained a Medical Discussion Paper entitled "Headaches" that was prepared by Dr. John Edmeads, neurologist in 2002. Dr. Edmeads was a Fellow of the Royal College of Physicians and Surgeons. He served the College as Chief Examiner in neurology from 1981 to 1983. He joined the University of Toronto faculty in 1967 and became Professor of Medicine in 1984. He led research in cerebral blood flow, chronic headache and medical education. He received numerous awards for his work and teaching in these areas. Dr. Edmeads was Head of Neurology at Sunnybrook Health Science Center from 1969 to 1993 and Head of the Department of Medicine and Physician in Chief at Sunnybrook Health Science Center from 1994 to 2001. Dr. Edmeads died in April 2007.

[67] Dr. D. Rowed reviewed the Discussion Paper in 2010, and is of the opinion that the paper still provides a balanced overview of the medical knowledge in this area.

[68] The introduction to the Discussion Paper indicated:

HEADACHE

Headache is not a disease. It is a symptom, with many causes, mostly benign (like migraine) and rarely ominous (like brain tumor). This discussion will focus on headaches encountered in the setting of the workplace – headaches associated with traumatic injury to the head and/or neck, and headaches related to exposure to toxic substances. As prologue, it is important to become familiar with the causes of headaches in general, and with the benign dysfunctional headaches (such as migraine and tension-type headaches) that so many people have as a result of hereditary, stress, or idiopathic (i.e. unknown) factors.

[69] Of relevance to these proceedings, the Discussion Paper went on to note:

Post-traumatic headaches

The headaches following trauma, in contrast, tend to be much more refractory, and are a frequent source of disability and compensation claims. Head injury itself is remarkably common, being reported in about two million people in the USA every year (about 1% of the population). Forty-five percent of these injuries are due to motor vehicle accidents, thirty percent to falls, twenty percent to occupational and recreational accidents, and the remainder to miscellaneous causes including violence. Eighty percent of these head injuries are deemed mild, ten percent moderate, and ten percent severe, using as an index of severity the conventional criterion of duration of post-traumatic amnesia (the length of time after the injury for which the person has no memory). The incidence (the number of people in a population who *develop* a condition within a given time span) of post-traumatic headaches is highly variable, depending on the make-up of the population studied, and ranges from 30 to 70%. Older people are more likely to develop post-traumatic headaches, as are people who have had previous benign dysfunctional

headaches such as migraine or tension-type headaches, and women. There is a paradoxical and controversial (see page 10) *inverse* relationship between the severity of the head injury and the likelihood of developing post-traumatic headaches. Some studies have shown that in societies (for example, a former Iron Curtain country) which do not have traditions of compensation for headaches or of private litigation, there is a much lower incidence of post-traumatic headaches than in neighboring NATO countries or in North America; these studies have provoked much emotionally-charged debate (see page 8 for a discussion of the controversy about physical versus psychological factors).

* * *

There is an emerging consensus on how to diagnose post-traumatic headaches. The International Headache Society (IHS) has published diagnostic criteria for post-traumatic headaches. The type of headache is first established by how well it conforms to the IHS criteria for tension-type headache, migraine, etc. The issue of how likely that headache is to be causally related to head trauma is then assessed by the post-traumatic criteria, which are paraphrased as follows:

Headache follows significant head trauma, the significance being demonstrated by at least one of:

1. Loss of consciousness occurred
2. Post-traumatic amnesia occurred, and lasted more than 10 minutes
3. At least two of the following tests showed relevant abnormalities:

Clinical neurological examination, skull X-ray, CT scan, MRI, evoked potentials, spinal fluid examination, vestibular function studies, neuropsychological testing.

Headache begins within 14 days of trauma.

If the headache disappears within 8 weeks following the trauma, it is termed “acute”; if it does not, it is termed “chronic”.

A headache that begins more than 14 days after the trauma is deemed unrelated to the trauma. A head trauma that does not meet the criteria for significant (“significant does not necessarily equate with “severe”, but rather with “not trivial”) may still be followed by headache and other symptoms, but the causal relationship is then less defined.

[70] The Panel was also provided with a Medical Discussion Paper entitled “Neck and Arm Pain and Related Symptoms: Cervical Disc Disease” prepared by Dr. J.F.R. Fleming, Professor Emeritus, Division of Neurosurgery University of Toronto, Division of Neurosurgery, Toronto Western Hospital University Health Network and revised February 2012 by: Dr. Joel Finkelstein, MD, MSc, FRCS(C) Associate Professor, Department of Surgery, University of Toronto Orthopaedic Surgeon, Sunnybrook Health Sciences Centre.

[71] The Discussion Paper notes:

Pain

Neck pain: Pain nerve endings are located in the various ligaments and muscles in the neck, as well as in the face and unco-vertebral joints and the outer layer of the disc (annulus fibrosis). When these structures are irritated, strained or inflamed, pain can be referred upwards to the base of the occiput or to between the shoulder blades. This is referred pain which needs to be distinguished from radicular pain which goes into the arms. Referred pain is axial neck pain and generally occiput to C3 is referred to the base of the occiput while C4 to C7 is referred between the shoulder blades. Neck pain, which is a very common symptom in the general population, is usually muscular or ligamentous in origin, and is usually self-limited although it can be persistent. The natural processes of healing of areas of inflammation result in improvement in almost all cases. In fact, the

pain from serious neck injuries such as fractures, dislocations and most cervical spine surgery almost always resolves after a few weeks or at most a few months. There is usually little if any correlation between neck pain and the degenerative changes so commonly seen on x-rays and scans. The role of the physician is to rule out other causes of neck pain. In the absence of physical findings, psychosocial factors may play a role and be barriers to recovery. Treatment must address factors such as depression, anxiety, fear and workplace.

[72] The Discussion Paper goes on to note:

Shoulder joint pain.

Pain from a degenerated or injured shoulder joint often mimics and may be mistaken for nerve root pain, as the pain often spreads down to the top of the arm, but not below the elbow. Shoulder joint pain may inhibit the patient's willingness to contract the arm or hand muscles strongly when these muscles are being tested for strength, thus leading to the erroneous conclusion that there is true muscle weakness caused by impairment of a nerve root.

The most common sources of shoulder pain are: 1) Rotator cuff tendonitis or tear; 2) osteoarthritis of the glenohumeral joint, and 3) Acromioclavicular joint (AC Joint) arthritis. Pain from a diseased or injured shoulder joint can be distinguished from nerve root pain by provocative testing: Rotator cuff tendonitis creates pain with active shoulder abduction above 30 degrees and impingement testing is positive. Osteoarthritis of the shoulder will have pain with active and passive movement of the shoulder usually in all planes; there may be crepitus and limited range of motion. AC joint arthritis will have tenderness directly over the joint and pain with forced adduction and rotation of the shoulder.

(ix) Analysis

[73] The appeal is allowed in part for the reasons set out below.

[74] The worker suffered injuries as a result of work-related falls that occurred on September 28, 2007 and December 2, 2007.

[75] The worker was diagnosed with a minor head injury and soft-tissue injuries following the fall at work on September 28, 2007. The worker continued to have headaches following the accident and was referred to a neurologist and underwent testing in the form of a CT scan and an MRI of her head.

[76] The worker fell at work on December 2, 2007. She was diagnosed with a sprain/strain of her low back and upper right extremity.

[77] The worker returned to work with the accident employer for a period of time in between the two accidents performing modified duties.

[78] The Panel notes that the worker received physiotherapy treatment for a period of time for her back and right shoulder.

[79] For reasons that will be explained below, the Panel concludes that the worker is entitled to ongoing entitlement for her head injury, including a NEL determination for her head.

[80] In coming to this conclusion, the Panel notes, in particular, that the worker testified that she lost consciousness after her fall on September 28, 2007. The Panel further notes that the worker testified that she continued to have headaches from the date of the accident forward. The Panel further notes that, according to Dr. Edmeads' Discussion Paper entitled "Headaches," there

is a consensus emerging with respect to headaches following significant head trauma with significant head trauma requiring one of three specified criteria. The Panel notes that one of the criteria for diagnosing a “serious” head injury has been met by the worker as she had a loss of consciousness following the fall.

[81] The Panel notes that *Operational Policy Manual* (“OPM”) Document No. 11-01-05 states:

Workers reach maximum medical recovery (MMR) when they have reached a plateau in their recovery and it is not likely that there will be any further significant improvement in their medical impairment.

[82] The OPM Document further states:

Significant improvement refers to a marked degree of medical improvement in the work-related impairment that is demonstrated by a measurable change in objective clinical findings.

Impairment means a physical or functional abnormality or loss (including disfigurement) which results from an injury and any psychological damage arising from the abnormality or loss.

Permanent impairment means impairment that continues to exist after the worker reaches maximum medical recovery.

[83] The Panel also notes that Dr. Kay commented in December 2008 that the worker’s headaches had persisted despite treatment consisting of Gabapentin and Nortriptyline. The Panel notes that this report was written over a year following the injury.

[84] For the foregoing reasons, the Panel allows the worker’s request for ongoing entitlement for her head injury, including entitlement to a NEL determination for her head. The Panel finds that the MMR date is December 3, 2008, the date of Dr. Kay’s aforementioned report.

[85] For reasons that will be explained below, the Panel is not persuaded that the worker is entitled to ongoing entitlement to benefits for her back, neck, and right shoulder, nor a NEL determination.

[86] In coming to this conclusion, the Panel notes that the worker was diagnosed with soft-tissue injuries as a result of her September 2007 fall. The Panel further notes that the worker was diagnosed with a “strain/sprain” of her right upper extremity following her December 2007 injury.

[87] The worker underwent physiotherapy but reported only minimal improvement in her pain symptoms in her back and right shoulder.

[88] The Panel notes that as of April 2008 when the worker was assessed by Dr. Bhardwaj, the worker had pain in her right shoulder, back and leg. Dr. Bhardwaj commented that the worker had pre-existing back issues that resulted in a CT scan of the worker’s lumbar spine in 2007. He also noted that the worker could return to work on a graduated basis.

[89] The worker’s representative submitted that there is no evidence that the worker’s injuries resolved. The Panel notes that there is no evidence before it that the worker required any treatment or restrictions for her right shoulder, neck, back or upper extremity beyond April 2008. The Panel acknowledges that the worker complained of ongoing pain, but there are no diagnoses noted in the materials beyond that of a “sprain/strain” or “soft-tissue injury.” The Panel relies on the Discussion Paper entitled “Neck and Arm Pain and Related Symptoms: Cervical Disc

Disease” which notes that ongoing pain can be correlated with specific diagnoses in the neck or shoulder such as a fracture of a neck vertebrae or a shoulder rotator cuff tear. The Panel notes that there is no evidence of significance before it of further testing, consultation or treatment required for the worker’s right shoulder, neck, back or right upper extremity.

[90] The Panel finds based on the preponderance of evidence that the worker is not entitled on an ongoing basis for benefits for her neck, right shoulder, or right upper extremity arising out of either of the slip and fall on September 28, 2007 or December 2, 2007.

[91] Lastly, the Panel finds for reasons that will be explained below, the worker is not entitled to any further LOE benefits.

[92] In coming to this conclusion, the Panel notes that OPM Document No. 18-03-02 states:

A worker who has a loss of earnings as a result of a work-related injury is entitled to payment of loss of earnings (LOE) benefits beginning when the loss of earnings begins. The payment continues until the earliest of

- the day on which the worker’s loss of earnings ceases
- the day on which the worker reaches 65 years of age, if the worker was less than 63 years of age on the date of the injury
- two years after the date of injury, if the worker was 63 years of age or older on the date of the injury, or
- the day on which the worker is no longer impaired as a result of the injury.

[93] The OPM Document goes on to state:

Payment of full LOE

Treatment with no return to work

If the nature or seriousness of the injury completely prevents a worker from returning to any type of work, the worker is entitled to full LOE benefits, providing the worker co-operates in health care measures as recommended by the attending health care practitioner and approved by the WSIB. If the worker does not co-operate, the WSIB may reduce or suspend the worker’s LOE benefits.

[94] The Panel finds that there is no evidence of significance that the worker was completely unable to work as of June 1, 2008. In coming to this conclusion, the Panel notes that Dr. Bhardwaj stated as of April 2008 that the worker could return to work, with restrictions. The Panel also notes that as of June 2008, Dr. Wang advised that the worker’s “disability persists” but provided no reasoning or quantification to substantiate this statement. The Panel also notes that there is no medical documentation before it from June 18, 2008 until 2013 when the worker sought treatment for depression. The Panel finds, therefore, that the worker was only partially disabled beyond June 1, 2008.

[95] The Panel acknowledges that the worker had ongoing headaches as of June 2008 that had persisted since September 2007, however, there is no evidence of significance before the Panel which would substantiate that the worker’s headaches completely prevented the worker from returning to the modified work that was offered by the employer. The Panel notes that the modified duties offered by the accident employer were light and did not violate the restrictions provided in November 2007 of standing for 15-30 minutes, sitting for 10-15 minutes, lifting up to 5 kgs on an occasional basis and pushing/pulling 10 lbs. on an occasional basis. The Panel further notes that the worker was not permitted to use a ladder, drive, work above shoulder level,

or bend/twist her neck on a sustained basis, and that the duties offered accommodated these restrictions. The Panel further finds that the employer offered the worker suitable modified duties as of May 2008 that alternated sitting and standing and provided a variety of different duties. The worker provided no evidence that she contacted the employer to discuss alternative duties.

[96] The Panel also notes that there is no evidence before it of significance that the worker's headaches presented restrictions that prevented the worker from performing the modified duties offered.

[97] The Panel concludes, therefore, that the worker is not entitled to LOE benefits beyond June 1, 2008 as she refused suitable modified duties, and, as such, any loss of earnings beyond June 1, 2008 arose from the worker's decision to refuse the suitable work and not her workplace injuries.

DISPOSITION

[98] The appeal is allowed in part as follows:

1. The worker is entitled to a NEL determination for her head with an MMR date of December 3, 2008;
2. Ongoing entitlement for the worker's back, neck and right upper extremity is denied;
3. The worker is not entitled to any LOE benefits beyond June 1, 2008.

[99] The nature and duration of benefits flowing from this decision will be returned to the WSIB for further adjudication, subject to the usual rights of appeal.

DATED: April 12, 2017

SIGNED: C.J. Ramsay, M. Lipton, S. Roth